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A Quest for Life and Its Meaning: Ageing in Sweden in the Context of Migration

Implications for Person-Centred Care in Later Life

Anna Zhou

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Abstract

Background
Sweden's population is ageing in a society shaped by long-term migration. Growing numbers of foreign-born adults are reaching later life after varying periods of residence in the country. Previous research has generated important knowledge about inequalities, access to services, and particular care settings, but research grounded directly in older foreign-born adults' own accounts of ageing remains relatively limited. Such knowledge is important for person-centred later-life care, which depends on understanding how individuals themselves experience ageing and what matters to them.

Aim
The overall aim of this thesis was to explore how older foreign-born adults living in Sweden experience ageing. By developing knowledge grounded in their own perspectives, the thesis also aimed to contribute to a better understanding of the conditions that support meaning, belonging, dignity, and well-being in later life, and thereby inform person-centred care in later life.

Design and method
The thesis comprises four qualitative studies informed by the Glaserian tradition of grounded theory. Data were collected in two sequential phases in a medium-sized municipality in Sweden. Phase I comprised 15 individual interviews. Phase II comprised five focus groups organised around shared language backgrounds and one individual interview, involving 23 participants. In total, 38 older foreign-born adults participated. Analysis proceeded iteratively through open and selective coding, constant comparison, memo-writing, theoretical sampling, sorting, and theoretical coding.

Findings
The overarching process was conceptualised as a quest for life and its meaning. Across the four studies, this quest was synthesised as an ongoing effort to preserve a meaningful self under changing

circumstances. Participants sought continuity by relating past experiences, present circumstances, and anticipated futures to one another. Ageing could involve bodily decline and increasing dependence, but also a broader perspective, greater acceptance, and appreciation of life. Migration sometimes complicated belonging and recognition, but could also provide additional experiences and frames of reference through which later life was understood.

Family was a central source of continuity, belonging, and a sense of accomplishment. Participants valued close relationships with children and grandchildren while generally wishing to avoid becoming a burden. Social connectedness beyond the family depended not simply on social contact, but on mutual understanding and recognition. Trust in Swedish society could coexist with experiences of not always being fully understood in interpersonal encounters.

Participants described autonomy and independence as important to maintaining dignity while they retained the capacity to manage everyday life. When dependency or loss of control became more salient, however, the conditions for sustaining dignity shifted. As personal agency weakened, dignity increasingly depended on whether family members, care professionals, and care systems could understand, respect, and safeguard the person's wishes, values, and ways of being.

Conclusions

The findings do not suggest that older foreign-born adults have fundamentally different concerns from other older adults. Rather, they show how concerns common to later life—meaning, belonging, recognition, autonomy, and dignity—may take particular forms when lives have unfolded across migration histories, languages, and more than one social world. The thesis highlights the importance of person-centred later-life care that attends to communication, continuity, meaningful relationships, and what matters to the individual, while

remaining responsive to migration-related circumstances without making assumptions based on group membership.

Keywords: ageing; belonging; dignity; gerontological nursing; grounded theory; meaning in life; migration; older foreign-born adults; person-centred care; social connectedness

Sammanfattning

Bakgrund
Sveriges befolkning åldras i ett samhälle präglat av långvarig migration. Allt fler utrikesfödda personer når högre åldrar efter att ha bott olika länge i Sverige. Tidigare forskning har bidragit med viktig kunskap om ojämlikhet, tillgång till vård och omsorg samt erfarenheter i specifika vård- och omsorgssammanhang. Forskning som utgår direkt från äldre utrikesfödda personers egna erfarenheter av åldrandet är däremot fortfarande relativt begränsad. Sådan kunskap är viktig för personcentrerad vård och omsorg, som förutsätter en förståelse av hur individer själva upplever sitt åldrande och vad som är viktigt för dem.

Syfte
Det övergripande syftet med denna avhandling var att undersöka hur äldre utrikesfödda personer i Sverige upplever åldrandet. Genom att utveckla kunskap med utgångspunkt i deras egna perspektiv syftade avhandlingen också till att bidra till en bättre förståelse av de villkor som ger förutsättningar för mening, tillhörighet, värdighet och välbefinnande senare i livet, med relevans för personcentrerad vård och omsorg.

Design och metod
Avhandlingen omfattar fyra kvalitativa delstudier med utgångspunkt i grounded theory enligt Glaser. Datainsamlingen genomfördes i två faser i en medelstor svensk kommun. Fas I omfattade 15 individuella intervjuer. I fas II deltog sammanlagt 23 personer i fem fokusgrupper, sammansatta utifrån gemensam språkbakgrund, samt i en individuell intervju. Totalt deltog 38 äldre utrikesfödda personer. Analysen genomfördes iterativt och omfattade öppen och selektiv kodning, ständig jämförelse, memoskrivande, teoretiskt urval, sortering och teoretisk kodning.

Resultat
Den övergripande processen formulerades som *en strävan efter livet och dess mening*. I en syntes av de fyra delstudierna framträdde denna strävan som ett fortgående arbete för att bevara en meningsfull känsla av vem man är under förändrade livsvillkor. Deltagarna skapade kontinuitet genom att knyta samman tidigare erfarenheter, sin nuvarande situation och föreställningar om framtiden. Åldrandet kunde innebära förändrade fysiska förutsättningar och ökat beroende, men också ett vidare perspektiv, större acceptans och ökad uppskattning av livet. Migration kunde försvåra tillhörighet och erkännande, men också ge ytterligare erfarenheter och referensramar för att förstå livet och åldrandet.

Familjen var en central källa till kontinuitet, tillhörighet och en känsla av att ha åstadkommit något i livet. Deltagarna värdesatte nära relationer till barn och barnbarn men ville i regel undvika att bli en belastning för dem. Social samhörighet utanför familjen handlade inte bara om social kontakt, utan också om ömsesidig förståelse och erkännande. Tillit till det svenska samhället kunde samexistera med erfarenheter av att inte alltid bli fullt ut förstådda i mellanmänskliga möten.

Deltagarna beskrev autonomi och självständighet som viktiga för att upprätthålla värdighet så länge de själva kunde hantera vardagen. När beroende eller minskad kontroll blev mer framträdande förändrades däremot villkoren för att upprätthålla värdighet. När den egna handlingsförmågan minskade blev förutsättningarna för att upprätthålla värdigheten i högre grad beroende av om familjemedlemmar, vård- och omsorgspersonal samt vård- och omsorgssystem kunde förstå, respektera och värna personens önskemål, värderingar och sätt att vara.

Slutsats
Resultaten tyder inte på att det som är viktigt senare i livet i grunden skiljer sig mellan äldre utrikesfödda personer och andra äldre

personer. Snarare visar avhandlingen hur aspekter som är centrala i åldrandet – mening, tillhörighet, erkännande, autonomi och värdighet – kan ta sig särskilda uttryck när livet har formats av migration, flera språk och erfarenheter från mer än en social värld. Avhandlingen synliggör betydelsen av personcentrerad vård och omsorg för äldre, där kommunikation, kontinuitet, meningsfulla relationer och det som är viktigt för individen uppmärksammas, samtidigt som migrationsrelaterade omständigheter beaktas utan att antaganden görs utifrån grupptillhörighet.

Preface

> "*It is in the mind of the marginal man that the conflicting cultures meet and fuse. It is, therefore, in the mind of the marginal man that the process of civilization is visibly going on, and it is in the mind of the marginal man that the process of civilization may best be studied.*"
>
> - Robert E. Park, 1928

When I arrived in Sweden from China at the end of 1999, I encountered a society that appeared, in many respects, strikingly different from the one I had previously known. As a newcomer, I was fascinated not only by the landscape and everyday customs, but also by how society was organised and by the norms and values that seemed to shape everyday life.

I began by reading widely about Sweden's history and development, alongside Swedish fiction, memoirs, and essays, while also following contemporary political, economic, and social policy debates. Over time, my initial fascination became more reflective. I became particularly interested in the legal system, the welfare state, and healthcare — institutions that may express a society's moral aspirations while also revealing its tensions, limitations, and blind spots.

Over the past two decades, I have come to know Swedish society from within through two professional paths that have shaped me deeply: as an authorised interpreter and through nursing, from early work in residential eldercare to clinical experience in surgical and cardiology care, followed by eight years in specialist geriatric nursing. As an interpreter, I have often stood at the boundary between languages, between individuals and institutions, and between different ways of

understanding the world. As a nurse in geriatric care, I have encountered ageing, vulnerability, and dignity in their most concrete forms.

The Sweden I first encountered has not remained unchanged. Globalisation, digitalisation, demographic change, and shifting political debates have placed Swedish values and institutions in new contexts. Robert E. Park was among the first migration scholars I encountered, and his reflections on living across cultural worlds have remained with me ever since. His term *the marginal man* belongs to another historical period, but I was deeply influenced by his broader insight that migration offers a distinctive vantage point from which both migrants' lives and the societies in which they live may be understood.

This thesis grew out of the meeting point between these different strands of my life: my own experience of migration, my long-standing curiosity about Swedish society, and my work as an interpreter and geriatric nurse. It is therefore both an academic project and part of a longer personal journey.

Above all, I am deeply grateful to the older people who shared their experiences with me so openly and with such trust. Their reflections on life and ageing form the foundation of this thesis. I hope that I have represented their accounts faithfully and respectfully.

Norrköping,
10 August 2026

Anna Zhou

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I am also deeply grateful to the Geriatric Clinic, which made this doctoral work possible by supporting my research time throughout these eight years while I continued my clinical work as a nurse. I am especially thankful to my colleagues at the clinic for their encouragement, understanding, and practical support. Their willingness to adjust schedules around research seminars and other academic commitments greatly facilitated the combination of clinical work and doctoral studies. Their moral support throughout these years has meant a great deal to me.

List of papers

I. Zhou A, Larsson Ranada Å, Roos S, Hellström I. Aging in Sweden as foreign-born—a quest for life and its meaning: a grounded theory study. *Quality in Ageing and Older Adults*. 2026;27(2):89–100. doi:10.1108/QAOA-10-2024-0061.

II. Zhou A, Hellström I, Roos S, Larsson Ranada Å. Aging with my family: a grounded theory approach on the role of family when aging as foreign-born. *BMC Geriatr*. 2024;24(1):45. doi:10.1186/s12877-023-04641-3.
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III. Zhou A, Larsson Ranada Å, Roos S, Hellström I. A quest for understanding: older migrants' wellbeing beyond integration in Sweden. *Front Public Health*. 2025;13:1620911. doi:10.3389/fpubh.2025.1620911.

IV. Zhou A, Hellström I, Roos S, Larsson Ranada Å. Living and dying in dignity among older migrants. *Nurs Ethics*. Published online 27 May 2026. doi:10.1177/09697330261449309.

Contributions

I initiated the four studies and developed the initial research questions, study designs, methodological approaches, and analytical focus in consultation with my supervisors. I had primary responsibility for study preparation, participant recruitment, data collection, and management of the empirical material.

I conducted the principal analyses, including coding, categorisation, interpretation, and synthesis of the findings. I prepared the initial drafts of all four manuscripts and had primary responsibility for subsequent revisions. My supervisors contributed methodological guidance, critical discussion, alternative perspectives, and detailed feedback that helped refine the analyses and manuscripts.

As corresponding author, I was responsible for manuscript submission, communication with the journals, coordination of revisions, and preparation of responses to reviewers in consultation with my supervisors.

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Chapter 1

Introduction

1.1 Ageing in a society shaped by migration

People reaching later life in Sweden today have increasingly diverse life histories. Many were born in another country and have spent part or most of their adult lives in Sweden. By the end of 2024, approximately one fifth of the Swedish population was foreign-born (1), and growing numbers are now entering later life after widely varying periods of residence in the country. Sweden's ageing population therefore increasingly includes people with diverse migration histories and life circumstances.

The older foreign-born population in Sweden is highly heterogeneous. It includes post-war labour migrants, predominantly from Finland and other European countries, and refugees and family migrants who arrived from the late twentieth century onwards from a broader range of regions, including the Middle East, Africa, Asia, and Latin America. Their circumstances vary according to country of birth, reasons for migration, age at migration, length of residence, socioeconomic background, family situation, and health status. Despite this diversity, older foreign-born adults are often approached in research, policy, and public discourse as if they constituted a single and self-evident group (2–3).

These developments concern more than demographic change. They raise questions about how later life is understood and experienced in the context of migration; how continuity, belonging, and dignity are sustained under changing circumstances; and how care and welfare institutions respond to people with diverse histories, circumstances, and ways of communicating.

1.2 Older foreign-born adults in Sweden: changing research perspectives

Research on older foreign-born adults in Sweden is not new. During the 1980s and 1990s, studies examined the living conditions of older adults from particular migrant groups, including Finnish, Bosnian, and Iranian migrants (4–8). These studies contributed important knowledge about social conditions, family relationships, and access to welfare. However, they generally focused on particular national or linguistic groups and reflected a period when Sweden's older foreign-born population was smaller and more strongly shaped by earlier migration cohorts.

From the 2000s onwards, the field broadened alongside changing migration patterns and growing policy interest in population ageing and diversity. Research increasingly addressed social and economic inequalities, health disparities, language barriers, access to services, care preferences, and the implications of demographic change for the welfare state and eldercare (9–12). This work generated important knowledge about structural disadvantage and institutional shortcomings but often approached older foreign-born adults in relation to vulnerability, dependency, unmet needs, or challenges faced by health and welfare services.

Alongside this predominantly structural and institution-oriented literature, a smaller body of qualitative research has examined older foreign-born adults' own experiences. Studies of public care services and transitions to residential care have highlighted the importance of independence, communication, family ties, and connections with earlier life (13–14). Other studies have addressed existential loneliness (15), identity negotiation in relation to old age and migrancy (16), the pursuit of vital goals and exercise of agency in seeking a good later life (17), and the role of meeting places in fostering belonging, trust, and social support (18).

Within Swedish nursing and healthcare research, this participant-centred perspective remains particularly limited. A small body of studies has addressed palliative and end-of-life care, mainly from professional or family perspectives, highlighting flexibility, individual wishes, continuity, and the risks of cultural stereotyping (19–21). These studies provide only limited insight into how older foreign-born adults themselves understand ageing outside specific care situations.

Overall, Swedish research has gradually broadened to give greater attention to older foreign-born adults' agency, identities, relationships, and lived experiences. Nevertheless, research grounded directly in their own accounts remains relatively limited and dispersed, particularly within nursing and healthcare.

1.3 Otherness, categorisation, and taken-for-granted knowledge

Historical research by Harrison (22) and Byström and Frohnert (23) shows that migration has long formed an integral part of Swedish history and contributed to the country's social, economic, and cultural development. During the twentieth century, however, immigration increasingly became defined as a distinct issue of public policy, political debate, and administration. Public authorities, welfare institutions, and public debate increasingly classified migrants through different categories, while which groups were regarded as different or problematic changed with the social and political circumstances of the time (22–23).

Against this background, a central point of departure for this thesis is that 'immigrant' is not merely a description of migration background. It is also a social category whose meaning is shaped by public discourse, institutional practices, and everyday processes of categorisation. Research indicates that in Sweden, the term invandrare ('immigrant') has often been positioned in contrast to Swedishness and may remain socially consequential long after

migration itself; people who have lived in Sweden for decades may therefore continue to be understood primarily through their immigrant background (24–27).

Such categorisation may also produce taken-for-granted knowledge about the people included within a category. Research has shown that assumptions about older migrants may acquire the status of established knowledge even when based on simplifications or stereotypes (3,28–30). These assumptions may concern family relationships, care expectations, dependency, or broader cultural and value orientations. When treated as self-evident, they may shape how an older person is understood before that individual's own experiences and perspectives have been heard. Torres (28) describes this process as 'otherness under construction', highlighting how categorisation can shape the ways in which older migrants are perceived and approached. Categories therefore do more than describe populations; they may influence what is noticed, which questions are asked, and what responses are anticipated.

1.4 Ageing, health, and selfhood in later life

Ageing involves changes in physical and mental capacities, but these vary considerably between individuals and are not determined by chronological age alone. Later life also unfolds through changes in roles, relationships, activities, and living circumstances. WHO's concept of healthy ageing (31) reflects this broader perspective by focusing on functional ability and the interaction between a person's capacities and their environment. Functional ability includes possibilities for making decisions, maintaining relationships, participating in society, and engaging in everyday life. Healthy ageing is therefore not equated with the absence of disease, but concerns the conditions that enable well-being despite changing capacities.

Nordenfelt's holistic theory of health (32–33) provides a complementary philosophical perspective. In this theory, health

concerns a person's ability, under standard circumstances, to realise the vital goals required for an acceptable level of well-being. The concept of *vital goals* is particularly relevant to the present thesis because it directs attention beyond disease and bodily function towards what a person needs to be able to pursue in order to live a life they value.

The notion of *standard circumstances* (32-33) is also important. In this thesis, such circumstances are understood as socially situated. Linguistic, relational, social, and institutional conditions may enable or constrain the pursuit of vital goals. For older foreign-born adults, these conditions may not always correspond to what is taken for granted in the surrounding society, while family networks, transnational ties, and experience across societies may also provide resources. Their significance, however, cannot be inferred from migration background alone.

From this perspective, selfhood becomes relevant because the goals people value are often bound up with their relationships, roles, autonomy, belonging, and sense of who they are. In the social-psychological tradition associated with Mead (34), the self is not understood as a fixed inner entity, but as developing and being sustained through social interaction. People come to understand themselves partly through taking the perspectives of others and through their participation in wider social worlds. Selfhood therefore involves both continuity in one's sense of who one is and an ongoing relationship with how one is understood and responded to by others. Changes in health or increasing reliance on others may consequently affect not only what a person can do, but also the conditions for maintaining continuity of self. Identity, recognition, and dignity become relevant because they concern whether a person can maintain a valued sense of self and be recognised as such under changing circumstances.

1.5 Person-centred care in a society shaped by migration

Swedish healthcare and eldercare are formally grounded in principles of human dignity, care on equal terms, self-determination, and respect for the individual, as expressed in the Health and Medical Services Act (35), the Patient Act (36), and national guidance on the value base of eldercare (37).

Person-centred care is an important principle in the care of older adults. It requires the person to be understood not only in terms of illness, diagnosis, or care needs, but as an individual with particular capacities, experiences, values, preferences, goals, and relationships. Participation, shared decision-making, and attention to what matters to the individual are therefore central (38-39).

In a society shaped by migration, person-centred care requires responsiveness to differences in life history, language, family relationships, and the meanings individuals attach to health and care. Reviews of healthcare for migrants have highlighted communication, participation, and attention to individual social and cultural circumstances as important conditions for person-centred care (40-41). Conversely, participation may be constrained when communication is inadequate, relevant relationships are overlooked, or cultural assumptions are allowed to substitute for dialogue with the individual.

These issues may become particularly important in later life, when frailty, cognitive impairment, or increasing dependence can affect communication and decision-making. Although Swedish healthcare places strong emphasis on individual self-determination (35–37), older adults´ wishes, values, and sense of self may also be sustained through relationships characterised by trust, shared history, and mutual understanding. This is consistent with a relational understanding of autonomy, in which autonomy is not exercised in

isolation but may be shaped and supported through relationships, particularly in situations of vulnerability (42).

Person-centred care in a society shaped by migration therefore requires responsiveness to migration-related circumstances without allowing assumptions about them to define the individual in advance. Equitable care involves creating conditions in which each person can participate, be understood, and remain connected to what matters to them.

1.6 Rationale for the thesis

The preceding sections point to a remaining knowledge gap. Swedish research has generated important knowledge about older foreign-born adults' living conditions, health, access to services, and experiences in particular social and care settings. Research grounded directly in their own accounts of ageing, however, remains relatively limited and dispersed. Studies have often focused on particular groups, settings, or aspects of later life, leaving less integrated knowledge about how different dimensions of ageing are experienced and related to one another.

This gap matters because migration-related social, linguistic, and institutional circumstances can shape opportunities for participation, access to care, communication, and continuity in later life. At the same time, a predominant focus on disadvantage, vulnerability, or service barriers may obscure older adults' agency, resources, and own interpretations of ageing. Research grounded in their accounts can therefore contribute to a more balanced understanding of both the conditions that constrain later life and those that support meaning, belonging, autonomy, and dignity. Such knowledge is important for person-centred and equitable care because it can help identify what matters to individuals without presuming particular needs on the basis of migration background.

Against this background, the thesis brings together four interrelated qualitative studies examining how older foreign-born adults understand and navigate ageing in Sweden. The studies address four interconnected dimensions that emerged as central in participants' accounts: meaning in life, family relationships, social connectedness, and dignity. By considering these dimensions in relation to one another, the thesis seeks to develop a more integrated conceptual understanding of ageing in a society shaped by migration and to consider its implications for person-centred care in later life.

Chapter 2

Aim of the Thesis

The overall aim of this thesis was to explore how older foreign-born adults living in Sweden experience ageing. By developing knowledge grounded in their own perspectives, the thesis also aimed to contribute to a better understanding of the conditions that support meaning, belonging, dignity, and well-being in later life, and thereby inform person-centred care in later life.

More specifically, the thesis seeks to:

- Explore perceptions and experiences of ageing among older foreign-born adults residing in Sweden (Study I).

- Explore how older foreign-born adults perceive the role of family as they age (Study II).

- Explore how older foreign-born adults in Sweden experience and navigate social connectedness as a determinant of well-being (Study III).

- Explore how foreign-born older adults in Sweden perceive and articulate dignity while ageing (Study IV).

Chapter 3

Methodology

3.1 Overall research design

This thesis used a qualitative research design based on the Glaserian tradition of grounded theory (43-46). The purpose was to generate a conceptual explanation of recurring patterns in how older foreign-born adults experienced and navigated ageing in Sweden, rather than to produce detailed accounts of individual lives. Consistent with the Glaserian approach, data collection and analysis followed the principle of emergence. Coding, constant comparison, memo-writing, and subsequent sampling decisions were informed by participants' concerns and by patterns emerging from the data. Engagement with substantive theories was deliberately delayed during the early analytical phase to reduce the risk of forcing the data into pre-existing concepts.

The project comprised two sequential phases of data collection and analysis. Phase I included Studies I and II and was based on individual interviews. Study I began as an open exploration of how older foreign-born adults understood ageing in Sweden. Through constant comparison, the core category a quest for life and its meaning gradually emerged. Study II then elaborated the role of family as an important dimension of this broader process. Phase II comprised Studies III and IV and included five focus-group discussions and one individual interview. These studies further examined analytically emergent dimensions concerning social connectedness as well as dignity in relation to dependency, vulnerability, and future care.

Individual interviews and focus-group discussions constituted the primary empirical material, while analytic memos and field reflections informed the developing analysis. Although the four

studies are analytically distinct, they are conceptually interconnected. Together, they reflect a progression from open exploration towards the successive elaboration and integration of dimensions of the emerging theoretical framework.

3.2 Sampling and participants

The research was conducted in a medium-sized municipality in eastern Sweden. The common inclusion criteria across the four studies were being born outside Sweden, being aged 60 years or older, residing in Sweden, and being able to participate in an individual interview or focus-group discussion, with language support where required. The lower age limit of 60 years was chosen in line with international ageing frameworks, in which people aged 60 years and older are commonly considered as an older population (47). It was used as a pragmatic sampling criterion rather than as an assumption that old age begins at a fixed chronological age. Most participants had migrated in adulthood, although age at migration ranged from 6 to 60 years. The sample therefore included considerable variation in both age at migration and length of residence in Sweden.

Sampling strategies differed between Phase I and Phase II in accordance with the evolving analytical focus of the project. Studies I and II drew on the same Phase I dataset, and Studies III and IV drew on the same Phase II dataset. Altogether, 38 individuals participated in the thesis research.

3.2.1 Phase I: Studies I and II

Studies I and II were based on 15 in-depth individual interviews with older foreign-born adults originating from different regions of the world. Participants were initially reached through convenience and network sampling (48), while selection was purposive (48) and aimed at obtaining variation in demographic, social and migratory backgrounds. Given practical limitations in recruitment, priority was

given to variation in country or region of birth, age, and gender. Variation in age at migration, length of residence, marital and living circumstances, educational background, employment history, and self-assessed health was also sought where feasible. The purpose was not to construct culturally representative subgroups, but to compare experiences across differing biographies and social circumstances.

After the first eight interviews, constant comparison indicated considerable convergence across participants with different social and migratory backgrounds. Participants generally expressed positive orientations towards life and ageing, valued resources acquired through experiences of more than one society and emphasised the importance of family and social relationships. Subsequent theoretical sampling (43,49) therefore sought participants whose circumstances might extend or challenge this emerging pattern, including people who had migrated later in life and whose limited Swedish proficiency, health problems, social circumstances, or dependence on relatives might restrict everyday participation.

To facilitate the inclusion of participants with limited Swedish proficiency, an amendment to the ethical approval was obtained allowing interviews with interpreting support. Two Spanish-speaking participants were subsequently recruited. Both had migrated to Sweden later in life and had limited Swedish proficiency, thereby extending variation in age at migration, language, health, and dependency. Their accounts helped refine the emerging pattern by showing that dependency and poor health could coexist with positive evaluations of later life and strong family support.

Recruitment and constant comparison continued until further interviews no longer substantially extended the properties and variation of the emerging categories. This point was regarded as theoretical saturation (43,45) rather than as evidence that all possible experiences had been captured.

Participant characteristics are presented in Table 1. Participants also varied in marital and living circumstances, employment status, educational background (from no formal schooling to university education), and self-assessed health (50) (from bad to excellent).

Table 1. Participant characteristics in Phase I (Studies I and II)

Participant	Age (years)	Gender	Age at migration to Sweden (years)	Country of birth
1	68	Female	38	India
2	75	Male	50	China
3	74	Female	38	Guatemala
4	64	Male	19	Turkey
5	85	Male	18	Finland
6	63	Male	27	UK
7	63	Female	30	Kongo
8	69	Female	33	Italy
9	78	Female	56	Peru
10	78	Male	58	Peru
11	63	Male	30	Chile
12	65	Female	17	Finland
13	60	Female	23	Romania
14	60	Male	33	Yugoslavia
15	71	Male	31	Bolivia

3.2.2 Phase II: Studies III and IV

Studies III and IV drew on a second dataset comprising five focus-group discussions and one individual interview. The focus groups included one Chinese-speaking, one Finnish-speaking, two Arabic-speaking, and one Spanish-speaking group. These language backgrounds were selected to provide variation across migrant communities with differing migration histories, periods of settlement, and social and linguistic circumstances in Sweden. Finnish- and Arabic-speaking populations represent comparatively large migrant

communities, while the Chinese-speaking group was intentionally included to broaden variation by representing a smaller community. The Spanish-speaking group contributed further variation in the migration histories represented in the sample.

Each group comprised between three and six participants. Methodological literature recognises the use of smaller or mini-focus groups, particularly when participants are highly involved in the topic or when complex or sensitive issues are discussed (51–54). Although recommendations regarding group size vary, groups of four or five have been described as mini-focus groups, and groups as small as three may still generate productive discussion when participants have substantial experience to contribute (51,54). Smaller groups can also provide greater space for individual contributions (51,53). These considerations were relevant in the present studies, which involved recruitment across several language backgrounds and discussions of ageing, dependency, future care, and dignity. The smaller format therefore provided greater opportunity for individual participation while retaining the interaction central to focus-group methodology.

To introduce variation within the Arabic-speaking sample, two groups were recruited from differing linguistic and social contexts. One comprised participants accustomed to Swedish-language settings, whereas participants in the other generally had shorter residence in Sweden and more limited Swedish proficiency and therefore participated with an interpreter. These differences were used as dimensions for comparison rather than as fixed indicators of integration.

Recruitment initially focused on eight municipal social centres for older adults but yielded no participants. The strategy was therefore broadened through collaboration with a Finnish social centre, a multicultural association, and a Syrian Orthodox church, which facilitated recruitment for the Finnish- and Arabic-speaking groups. Participants in the Chinese- and Spanish-speaking groups were

recruited through convenience sampling followed by network or snowball sampling (48).

Table 2. Participant characteristics in Phase II (Studies III and IV)

Participant	Age (years)	Gender	Age at migration to Sweden (years)	Country of birth
Chinese-speaking focus group (conducted in Mandarin)				
C1	69	Female	31	China
C2	63	Male	23	China
C3	62	Female	25	China
C4	66	Female	48	China
Individual interview (conducted in Mandarin)				
C5	81	Female	35	China
Arabic-speaking focus group 1 (conducted in Swedish)				
A1	62	Male	45	Iraq
A2	62	Male	45	Iraq
A3	62	Female	46	Iraq
A4	64	Female	19	Syria
A5	60	Female	13	Turkey
A6	60	Male	37	Iraq
Finnish-speaking focus group (conducted in Swedish)				
F1	62	Female	6	Finland
F2	85	Female	30	Finland
F3	85	Male	30	Finland
F4	69	Female	19	Finland
F5	78	Female	29	Finland
Arabic-speaking focus group 2 (conducted with Swedish–Arabic interpreter)				
A7	64	Female	45	Iraq
A8	68	Female	60	Iraq
A9	64	Male	47	Iraq
A10	67	Male	49	Iraq
Spanish-speaking focus group (conducted in Swedish)				
S1	67	Female	24	Chile
S2	76	Male	39	Chile
S3	68	Female	22	Chile

One 81-year-old Chinese-speaking participant preferred an individual interview to participation in a group. This was accommodated and also broadened variation within the Chinese-speaking sample in relation to age and later-life caregiving experience. The interview was retained as a supplementary comparative source.

In total, 23 individuals participated in Phase II. Participant characteristics are presented in Table 2.

3.3 Data collection

3.3.1 Phase I: Studies I and II

Phase I comprised 15 individual interviews conducted between January and September 2021. The interviews were open-ended and guided by a flexible interview guide. Each began with the broad question: 'What do you think about ageing in Sweden?' Follow-up questions and prompts were used to clarify participants' accounts and encourage them to elaborate on experiences they regarded as important. This open format supported the identification of participants' main concerns in accordance with grounded theory methodology.

Fourteen interviews were conducted face to face: eight in participants' homes and six in public locations agreed with the participants. One interview was conducted by Zoom at the participant's request. The interview with the Mandarin-speaking participant was conducted directly in Mandarin by the author, who is a native Mandarin speaker and an authorised and practising Mandarin–Swedish interpreter. In two interviews, Spanish-speaking participants were assisted by a relative who was an authorised interpreter.

The interviews lasted between 45 and 75 minutes and were audio-recorded with participants' consent. Interviews conducted in Swedish were transcribed verbatim in Swedish. The Mandarin interview was

transcribed and translated into Swedish by the author. In the interpreter-mediated interviews, both the participant's speech and the Swedish interpretation were audio-recorded, and the Swedish interpretation formed the basis of the transcript used in the analysis.

Following each interview, the author wrote field notes and an analytical memo documenting the interview context, relevant observations, initial comparisons, and emerging conceptual ideas. These memos informed subsequent data collection and analysis.

3.3.2 Phase II: Studies III and IV

Phase II comprised five focus-group discussions and one individual interview conducted between October 2024 and February 2025. Focus groups were selected as the principal data-collection method because interaction between participants was itself relevant to the aims of Studies III and IV. Participants could affirm, question, qualify, and elaborate one another's accounts, making shared understandings, differences, and points of tension more visible (52,54). This was particularly useful for exploring social relationships, belonging, future care, and dignity.

Each focus-group meeting comprised two consecutive thematic discussions separated by a short break and moderated by the author. The first addressed social connectedness and belonging, corresponding to Study III, and opened with the broad question: 'Who can you turn to when you need to talk or ask for help?' To facilitate reflection, participants were also given a diagram inspired by Antonucci's Convoy Model of Social Relations (55) and invited to indicate the people they had around them and describe these relationships. The diagram was used as an elicitation aid rather than as an analytical framework, and the discussion was allowed to develop according to participants' own experiences and concerns. The second discussion addressed dependency, future care, and dignity, corresponding to Study IV, and opened with the question: 'What does dignity mean to you?' Open follow-up questions were used to develop

the discussion in both sessions. Each part lasted approximately 45–60 minutes. Social connectedness and belonging were discussed first, before moving to the potentially more sensitive issues of dependency, future care, and dignity. The individual interview followed the same thematic sequence.

All focus-group discussions and the individual interview were moderated by the author. The Mandarin-language focus group and individual interview were conducted in Mandarin. The first Arabic-speaking, Finnish-speaking, and Spanish-speaking groups were conducted in Swedish. The second Arabic-speaking group was conducted with the assistance of an authorised Swedish–Arabic interpreter.

All discussions were audio-recorded with participants' consent. Swedish-language discussions were transcribed verbatim in Swedish. The Mandarin-language material was transcribed and translated into Swedish by the author. For the interpreter-mediated Arabic discussion, both the original-language contributions and the Swedish interpretation were recorded, and the interpreted Swedish speech formed the principal transcript used for analysis.

Following each focus group and the individual interview, the author wrote field notes and analytical memos concerning the interaction, group dynamics, emerging comparisons, preliminary conceptual ideas, and subsequent data collection plans.

3.4 Data analysis

All four studies were analysed using procedures derived from classical grounded theory as articulated by Glaser (43-46). Data collection and analysis proceeded iteratively through open and selective coding, constant comparison, theoretical sampling, memo-writing, theoretical saturation, sorting, theoretical coding, and integration. Although described separately below for clarity, these procedures

overlapped and informed one another throughout the analytical process. The author conducted the primary coding and memo-writing, while emerging categories and their relationships were discussed regularly with the supervisory team.

The analysis became progressively more selective as the project developed. The Phase I material was initially analysed openly for Study I. When family emerged as a significant dimension, the interviews were revisited comparatively for Study II. In Phase II, the two thematic parts of the material were analysed separately in relation to Studies III and IV, while comparisons were also made across the material where analytically relevant.

3.4.1 Open coding and constant comparison

Initial analysis involved line-by-line and incident-by-incident open coding (43,49), asking what participants were expressing, doing, resolving, or attempting to maintain. Codes remained close to the empirical material while incidents were continuously compared for similarities and differences. Constant comparison (43,49) continued throughout the analytical process, with incidents compared with other incidents and with emerging codes, categories, and their properties. For example, incidents concerning 'having a shoulder to lean on', 'always being together', and 'being a link between generations' were compared across interviews and contributed to the conceptualisation of family as a source of continuity and belonging in later life.

3.4.2 Selective coding, theoretical sampling, and saturation

As a core category began to emerge within each study, coding became increasingly selective. In Glaser's terminology, selective coding involves delimiting further coding to the core category, related categories, and their properties rather than continuing to code all aspects of the material (43,45).

Selective coding (43,45) was closely linked to theoretical sampling (43,49). As described in Section 3.2, subsequent sampling decisions were guided by the emerging analysis and directed towards participants and circumstances that could extend, challenge, or further develop the emerging categories (43,49). In Phase II, further groups also provided opportunities to compare emerging categories across differing linguistic and social circumstances.

Comparison included not only the content of individual contributions but, in Phase II, also the interaction through which participants confirmed, challenged, qualified, or extended one another's accounts. For example, in Study III, accounts of social distance were compared with expressions of trust in Swedish institutions and descriptions of Sweden as home. These contrasts helped differentiate interpersonal experiences of being understood from broader societal trust and belonging.

Selective coding (43,45) also involved reconsidering tentative categories. For example, migration as a lifetime event was not retained as a separate category because migration was found to permeate interpretations of past, present, and future rather than constitute a distinct domain.

Theoretical saturation (43,45) was assessed by considering whether additional data contributed new properties, variation, or relationships to the developing categories. In Phase II, after analysis of the Mandarin-speaking, first Arabic-speaking, and Finnish-speaking groups, the central categories and their principal relationships were sufficiently developed to guide subsequent comparison. The remaining groups added variation and tested the developing categories but did not substantially extend their properties or alter their principal relationships. Within the scope of these studies, this was taken to indicate theoretical saturation.

3.4.3 Memo-writing, sorting, and theoretical coding

Memo-writing accompanied all stages of the analysis and provided a flexible analytic space for developing comparisons, questions, and emerging conceptual ideas (43,45). Memos were repeatedly revisited, compared, and elaborated as the analysis progressed.

As categories matured, memos were sorted to clarify relationships among substantive categories (43,45). In Glaserian terminology, theoretical coding concerns the conceptualisation of relationships among substantive categories and their integration around the emerging core process (43,46). For example, in Study IV, categories concerning prerequisites for dignity, maintaining dignity, letting go, and surrendering with trust were related as dimensions of the overarching process of *negotiating dignity between control and connection*.

3.4.4 Theoretical integration and engagement with literature

Through sorting and theoretical coding, the developed categories and their relationships were integrated into coherent substantive theoretical accounts (43,45–46). Focused engagement with literature directly relevant to the emerging categories was delayed until the core categories and principal relationships had become analytically clear, reducing the risk that pre-existing frameworks would determine the emerging analysis (43–45). Relevant theoretical and empirical literature was then used to compare, illuminate, and further develop the concepts generated from the data.

Regular discussions with the supervisory team were used to question assumptions, consider alternative conceptualisations, and examine whether categories remained adequately grounded in the data. These discussions contributed to refinement and theoretical integration, while primary coding and memo-writing remained the responsibility of the author.

3.5 Ethical considerations

The studies were approved by the Swedish Ethical Review Authority (Dnr 2020-05672 for Phase I, amendment 2021-00955 for interpreter use, and 2023-04950 for Phase II).

Participants received oral and written information about the purpose and procedures of the research, the voluntary nature of participation, confidentiality, and their right to withdraw without giving a reason. Written informed consent was obtained before data collection. Participants could decline to answer individual questions, request a pause, or end an interview or discussion at any time.

Questions concerning ageing, social isolation, dependency, future care, and dignity could evoke sensitive experiences. Interviews and focus groups were therefore conducted flexibly and with attention to participants' comfort. In Phase II, social connectedness, belonging, and understanding were discussed before the more sensitive topics of dependency, future care, and dignity. This sequence allowed rapport to develop before participants were invited to reflect on potentially difficult aspects of later life.

Language support was considered important for equitable participation. Interpreter-mediated interviews and discussions enabled inclusion of participants who might otherwise have been excluded because of limited Swedish proficiency. Participants were informed about the interpreter's role and consented to the language arrangements. The potential influence of interpretation on data generation and meaning is considered in the methodological discussion.

Audio recordings and transcripts were pseudonymised and handled in accordance with institutional data-protection procedures. Identifying information was removed from transcripts and publications, and quotations were selected and presented with

attention to the risk that combinations of biographical details could make individual participants recognisable.

Chapter 4

Findings

4.1 An overview of findings across the four studies

This thesis comprises four qualitative studies that together illuminate how older foreign-born adults in Sweden experience and make sense of ageing. Study I identified the overarching process a quest for life and its meaning. Studies II–IV subsequently elaborated three interconnected dimensions of this process: family relationships, social connectedness, and dignity under conditions of vulnerability and dependency.

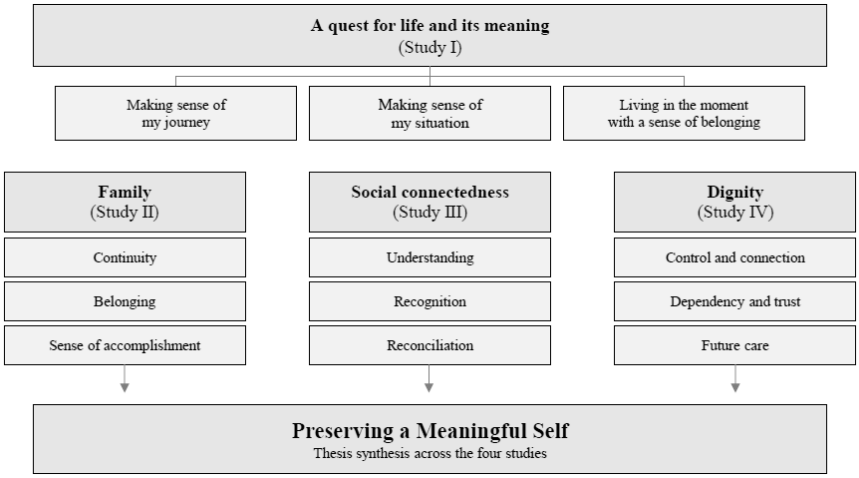


Figure 1. Overview of findings across the four studies

As shown in Figure 1, Study II illuminated how family contributed to continuity, belonging, and a sense of accomplishment. Study III showed how social connectedness and well-being were closely related to mutual understanding, recognition, and reconciliation. Study IV brought dignity into focus by showing how it was negotiated between control and connection as dependency and future care became more salient. In this context, care concerned not only practical support, but

also whether participants could trust others to understand, respect, and safeguard what mattered to them when their own agency weakened.

Across the four studies, these findings were synthesised at thesis level as an ongoing effort to preserve a meaningful self under changing bodily, relational, and social conditions. Participants' accounts of well-being were closely connected to meaning, belonging, recognition, valued relationships, and the possibility of retaining dignity. Questions of care became particularly important when participants anticipated greater dependence on family members, professionals, and care services.

4.2 Study I: A quest for life and its meaning

Study I provides the conceptual foundation for the thesis by identifying an overarching process through which participants experienced and oriented themselves towards ageing. The analysis suggested that participants navigated ageing through an ongoing process of meaning construction, captured by the core category a quest for life and its meaning. This quest involved weaving past, present, and future into a coherent and purposeful narrative and, in this way, supporting a positive sense of self in later life.

4.2.1 Making sense of my journey

Participants frequently reflected on where they came from, what they had done, and who they had been. Memories of parents, family of origin, migration, work, sacrifice, and parenthood helped them interpret their lives as meaningful and worthwhile. Migration was often described not only in terms of hardship, but also as a life journey that had made them wiser and stronger. Looking back in this way supported continuity and a sense that life still "added up". The past

was therefore not merely remembered but actively used to support continuity of identity in later life.

4.2.2 Making sense of my situation

Participants described ageing as creating a new awareness that life was finite and that the present should be used well. This gave rise to active efforts to manage later life through taking care of health, securing finances, doing what they enjoyed, spending time with people who mattered, and preparing themselves mentally and practically for the future. Ageing was described as both gain and loss: a mental win in terms of perspective, appreciation, and life wisdom, but also a period of physical decline and increasing vulnerability. Their accounts thus combined acceptance with agency.

4.2.3 Living in the moment with a sense of belonging

The outcome of this process was a present-oriented way of living marked by enjoyment, belonging, and continuity. Participants described pleasure in everyday activities, strong attachment to family, and a sense of living with "double assets" by combining resources from both their country of origin and Swedish life. Many expressed satisfaction, gratitude, and a sense that Sweden had become home. At the same time, future concerns remained, especially fear of losing independence and becoming dependent on others. Study I therefore showed that ageing as foreign-born in Sweden was experienced not simply as decline, but as an active and negotiated quest to sustain meaning, belonging, and a positive sense of self.

4.3 Study II: Family as a lifelong moral and relational context of ageing

Study II deepens the family arena suggested in Study I and shows that family operated as far more than a practical support network. It functioned as a central source of solidarity, identity, continuity, meaning, and life satisfaction in later life. The core category ageing

with my family captured how family remained an essential framework through which participants understood themselves, their life course, and what made ageing worthwhile. Family was not only where support was found. It was also where values were inherited, identities were confirmed, and life was experienced as coherent across generations. This was expressed through four interrelated dimensions concerning the family of origin, the life-mate, children and grandchildren, and the extended family.

4.3.1 Living with the heritage of my original family

Ageing evoked strong memories of parents and families of origin. Participants reflected on their childhoods, their upbringing, and the values they had received early in life. These memories connected ageing to biological, social, and cultural heritage. Family solidarity was described as something inherited from the original family and something participants wished to preserve and pass on. Many spoke about respect for older people, hard work, gratitude, and a sense of community as enduring values that had shaped their lives and that they continued to regard as important in later life.

For several participants, these family values were also part of what they felt distinguished them as foreign-born from native Swedes. Family was described not only as a private sphere, but as a moral orientation that had travelled with them through migration. Looking back on parents and earlier family life also made ageing feel more real. Some said that they began to understand their own ageing more deeply when their parents became frail, cognitively impaired, or died. In this way, memories of the original family linked later life to continuity, heritage, and reflection on what truly mattered.

4.3.2 Living with my life-mate

Long-term marital relationships emerged as especially important in the study. For those who had lived with a spouse for decades, the life-mate was described not only as a partner, but as a best friend,

companion, source of strength, and witness to the shared life course. Participants often spoke in terms of "we" rather than "I," reflecting how deeply everyday life, history, and future plans had become intertwined. Their spouses were the people with whom they shared routines, worries, joys, and ageing itself.

These long relationships also appeared to validate identity over time. Spouses knew one another across decades of migration, work, family-building, hardship, and adaptation. In that sense, the intimate relationship served as a source of continuity of self and often as a source of personal growth, mutual support, and self-esteem. At the same time, this closeness could also imply vulnerability, since life in older age was experienced as deeply bound up with the continued presence of the other.

4.3.3 Living with my children and grandchildren

Children and grandchildren were central to how participants understood the meaning of their lives. Many described migration itself as a family project undertaken not only for themselves, but for the sake of their children's future. Pride in children was often expressed as proof that sacrifice, hard work, and adaptation had been worthwhile. Parenthood was associated with accomplishment, responsibility, and satisfaction with the life lived.

Grandparenthood added a further layer of meaning. Participants described grandchildren as carrying hope, coherence, and meaning beyond their own lives. They wanted not only to love them, but to spend time with them, teach them things, and pass on values, perspectives, and skills. In this way, grandparenthood became a way of remaining present in the future generation and of linking the family's past with its future.

At the same time, participants generally did not want to burden their children with heavy responsibility. What they sought from their children in later life was often less practical care than emotional

closeness, affection, contact, and continuing belonging. They wanted to be remembered, visited, called, and included, while still trying to remain as independent as possible. This tension is important, because it shows that family support in later life was not imagined simply as dependence, but as a balance between closeness, recognition, and self-respect.

4.3.4 Living among my extended family

The study also broadened the meaning of family beyond the household and beyond immediate kin. Participants often expressed a wide understanding of family as community, solidarity, and belonging. Some described close relations with siblings, relatives, and friends in their countries of origin, even after decades in Sweden. Others emphasised friendship, religious communities, or wider social networks as part of the "extended family" of later life.

Several participants connected this broad sense of family to a moral ideal of how society itself should function: with more solidarity, gratitude, interdependence, and care between generations. Family in this wider sense became a metaphor for belonging in the world. It was also tied to the wish to give back, to remain useful, and to contribute to others in later life. In this way, the study showed that family was not only a set of kinship ties, but a moral and social world through which older foreign-born adults understood both their past and their ageing selves.

4.4 Study III: Social connectedness as a quest for understanding

Study III deepens the second arena of the thesis by showing that social connectedness in later life was not only a matter of having company or social activities, but of being able to understand others and to be understood by them. The core category, a quest for understanding,

captured participants' efforts to seek validation, make sense of Swedish society and their own changing identities, and ultimately reconcile aspirations with lived reality. The analysis comprised three interconnected dimensions: wanting to be understood, wanting to understand, and reaching for reconciliation. Social connectedness was thus closely tied to well-being, belonging, and the possibility of remaining socially intelligible in later life.

4.4.1 Wanting to be understood

Participants across groups emphasised the importance of relationships in which they could be understood in depth, particularly by people who shared their language, life experiences, or cultural background. Compatriots were often described as those most capable of grasping not only what participants said, but what they meant and felt. At the same time, many described difficulties in forming similarly meaningful relationships with native Swedes. These difficulties were linked to language barriers, cultural distance, reserved forms of Swedish sociality, and, in some groups, experiences of prejudice and exclusion.

Important differences appeared between groups. Finnish-speaking participants generally did not perceive themselves, or were not perceived by others, as immigrants in the same way as other groups. Spanish-speaking participants described relatively good integration and many contacts with Swedes, yet still experienced themselves as culturally distinct and sometimes not fully understood. Chinese-speaking participants described the greatest social isolation, emphasising both cultural distance from Swedish society and weak cohesion within the local Chinese community. Arabic-speaking participants described strong solidarity within their own communities, but also persistent difficulty being recognised and accepted in Swedish society despite long residence and contribution.

At the same time, limited interpersonal recognition did not mean that participants rejected Sweden or felt detached from it as a society.

Many expressed trust in Swedish institutions and appreciation of the security, order, and welfare the country offered. The findings therefore suggest a tension between societal trust and personal non-recognition: participants could feel confidence in Sweden as a society while still not always feeling fully understood within it.

4.4.2 Wanting to understand

The second analytical dimension concerned participants' wish to understand the society in which they had aged. This involved reflecting on Swedish norms, institutions, and ways of relating, but also on their own cultural backgrounds and identities. Arabic-speaking participants, for example, reflected critically on segregation, stereotypes, and exclusion, while also expressing strong appreciation for Swedish law, welfare, and public institutions. Chinese-speaking participants highlighted the difficulty of navigating Swedish bureaucracy and saw mutual help among compatriots as essential for making Swedish society more understandable. Finnish- and Spanish-speaking participants focused more on preserving and interpreting their heritage cultures while at the same time living with a relatively high degree of bicultural familiarity.

This dimension shows that understanding was not one-sided. Participants did not only want recognition from others; they also engaged in active interpretive work themselves, trying to comprehend both their adopted society and their own transformed lives within it. Several also described Sweden as home, not only because of long residence and practical security, but because it had become the homeland of their children and grandchildren. Social connectedness therefore involved reflection, comparison, and cultural interpretation as much as emotional support. It also involved negotiating a complex sense of belonging in which trust, attachment, and home could coexist with experiences of distance, misunderstanding, and incomplete recognition.

4.4.3 Reaching for reconciliation

The third analytical dimension concerned reconciliation. Participants described an ongoing effort to balance their ideals and wishes with the realities of ageing and migration. This included accepting that full inclusion in Swedish society might remain elusive, that younger generations might not carry forward their language and culture in the same way, and that social relationships sometimes required compromise. Reconciliation did not mean passive resignation. Rather, it reflected an active process of negotiating cultural difference, generational distance, relational limits, and one's own evolving identity.

Participants across groups also described reconciliation in more existential terms. Some let go of expectations that descendants would fully inherit their cultural legacy, while taking comfort in their children's and grandchildren's successful integration into Swedish society. Others spoke of solitude not only as isolation, but as a chosen and meaningful state that protected dignity and allowed reflection. The ideal social relationship was often described as balanced: preserving autonomy while remaining connected to others and seeking belonging without losing oneself. In this way, the study shows that well-being in later life depended not on complete integration or complete cultural continuity, but on reaching a workable reconciliation between them.

4.5 Study IV: Dignity as a negotiation between control and connection

Study IV brought dignity into explicit conceptual focus as an ongoing negotiation between control and connection. Participants associated dignity with autonomy, self-respect, continuity of self, and the ability to remain someone whose voice still counted. At the same time, dignity depended not only on self-determination, but also on recognition, belonging, trust, and the presence of others who could safeguard the person when personal agency weakened. The analysis

comprised four interconnected dimensions: prerequisites of dignity, maintaining dignity in everyday life, the art of letting go, and surrendering with trust.

4.5.1 The prerequisites of dignity

The first analytical dimension concerns what participants regarded as necessary for dignity to be possible in later life. A central theme was the cognitive threshold. Participants often described their ability to maintain or defend dignity as contingent on being able to think clearly, judge, remember, and remain aware of oneself. The prospect of dementia or severe cognitive decline was therefore especially frightening, not only as a medical condition, but as a threat to selfhood itself. What was feared was not simply forgetfulness but reaching a threshold beyond which they feared dignity could no longer be personally defended or even meaningfully experienced.

Autonomy was also described as a cornerstone of dignity. Participants repeatedly emphasised the importance of deciding over their own lives and not being spoken over or treated as incapable by either family members or professionals. Dignity depended on remaining a morally adult person whose judgement and preferences still mattered.

At the same time, the study showed that dignity was not understood only as something internally possessed. When selfhood weakened, dignity became increasingly dependent on connection as safeguard. In such situations, it was family members, care professionals, and institutions that had to uphold dignity through recognition, respect, and ethical conduct. Beyond a certain point, dignity was no longer only enacted by the person, but had to be relationally protected by others.

4.5.2 Maintaining dignity in everyday life

The second analytical dimension concerns the everyday work through which dignity was sustained while agency was still largely intact.

Within this dimension, one important theme was the sovereignty of 'now'. Many participants described living one day at a time, not as passive resignation, but as an active way of preserving control, perspective, and peace of mind in the face of an uncertain future. The present was the time domain in which they could still act, choose, and shape daily life.

Another theme was home as locus of identity. Home was described not merely as a place to live, but as a space of familiarity, authorship, language, and continuity. To remain at home for as long as possible meant preserving routines, preferences, language, and a sense of being oneself. For some participants, particularly when considering cognitive decline, a familiar language environment was seen as important for maintaining recognition, familiarity, and emotional security.

Dignity was also maintained through meaningful engagements. Participants described the importance of having something to do, someone to care about, or some way of remaining connected to a responsive world. These engagements varied widely — nature, neighbours, hobbies, pets, community solidarity, family contact — but what mattered was that they preserved purpose, rhythm, recognition, and a sense of being socially alive. Dignity in everyday life thus depended not only on autonomy in the abstract, but on the lived possibility of continuing to act, choose, and belong.

4.5.3 The art of letting go

The third analytical dimension captured the point at which maintaining full control was no longer possible and dignity had to be renegotiated through adaptation, acceptance, and moral boundary-setting. Participants described this as a process of reconciling ideals with new realities. One example concerned changing expectations of family care. Rather than insisting that adult children should care for them in traditional ways, many reformulated their expectations in light of migration, Swedish welfare norms, and the contemporary

lives of their children. In this way, letting go became a strategy for preserving both self-respect and relational balance.

This dimension also included deeper existential work. This dimension also included deeper existential work. Participants described gratitude and acceptance as ways of coming to terms with bodily decline, mortality, and circumstances that could no longer be controlled. In some accounts, forgiveness also appeared as part of this broader process of letting go, reflecting on what in life could be released, accepted, or reconciled with. Letting go did not mean giving up dignity. Rather, it meant adjusting to realities that could no longer be changed while preserving self-respect and a sense of inner coherence.

A particularly charged issue concerned weighing life against suffering. Some participants distinguished between living with limitations and living in a state of suffering they regarded as no longer compatible with dignity. Here, dignity was tied to the moral limits of what could still be experienced as a life worth living. In some groups, especially among Arabic-speaking participants, suffering was more explicitly framed through religion, faith, endurance, and God. In such accounts, suffering did not necessarily destroy dignity; it could also be transformed by the attitude taken toward it. These accounts showed that dignity in later life could be negotiated not only practically, but also morally and existentially.

4.5.4 Surrendering with trust

The final analytical dimension captured situations in which dignity could no longer be sustained primarily through personal control, but through a conscious surrender grounded in trust. Here, dignity became dependent on confidence that one would be cared for, recognised, and not abandoned when agency waned.

Participants described different sources of trust and protection. Some relied primarily on family members or close friends as human

safeguards, while others also expressed confidence in Swedish health and social care. Yet this trust was rarely unconditional. Physical safety alone was not enough. Participants wanted to be understood in their own language, habits, values, and cultural ways of being. Language and culturally responsive care were therefore prominent in participants' reflections on future care. Dignity depended not only on being cared for, but on being cared for as oneself.

The study also showed that surrender extended to readiness for the end of life. Participants described different ways of approaching death: some through faith, some through practical preparation, some through a more philosophical acceptance of death as natural. Despite these differences, surrender did not appear as passive defeat. Rather, it was an active, meaning-laden orientation toward dependency and death, made possible when trustworthy relationships, care structures, or existential frameworks allowed the person to let go without fear of abandonment. In this sense, dignity in later life was negotiated not only through holding on to control, but also through discovering when and how it was possible to relinquish control with trust.

4.6 How the four studies relate to each other

The four studies can be understood as successive elaborations of the same overarching process. Study I identified the existential orientation of ageing as a quest for life and its meaning. Study II showed how this quest was embedded in family relationships that provided continuity across the life course and between generations. Study III extended the analysis beyond the family by showing that social connectedness depended not simply on participation, but on understanding, recognition, and the negotiation of belonging. Study IV showed how questions of dignity became increasingly explicit when control, independence, and future care were considered.

Taken together, the studies suggest that preserving a meaningful self involved both continuity across time and recognition within relationships. Meaning, belonging, recognition, identity, and dignity

were therefore not separate concerns, but interconnected aspects of how participants understood and navigated later life. As personal agency became less certain, the relational conditions through which the person could remain understood, recognised, and treated with dignity became increasingly salient.

These findings are relevant to gerontological nursing and person-centred later-life care because they foreground concerns that emerged from participants' own accounts of ageing and draw attention to dimensions of later life that extend beyond functional ability and practical care needs. The implications of these findings for person-centred care are considered further in Chapter 5.

Chapter 5

Discussion

The following discussion interprets the findings across the four studies in relation to previous research and relevant theoretical perspectives. It focuses on meaning-making in later life, the relational conditions of ageing, dignity under increasing dependency, the significance of culture and language, and implications for person-centred later-life care.

5.1 Ageing as a quest for meaning in the context of migration

The findings suggest that later life involved an ongoing concern with whether life felt coherent, meaningful, and worthwhile. Participants did not describe ageing simply as something that happened to them. Rather, they engaged in ongoing interpretive work through which past experiences, present circumstances, and future prospects were brought into relation with one another. Migration, work, sacrifice, family life, and earlier hardships were revisited and reinterpreted in ways that allowed participants to understand their lives as meaningful wholes. This meaning-making was not only retrospective; it also shaped how they approached the present and anticipated the future.

Ageing was not described only in terms of loss. Participants often contrasted bodily decline with gains in perspective, acceptance, experience, and life wisdom. Their generally positive orientation towards ageing did not depend on denying physical change, but often on a sense of having lived, endured, contributed, and accomplished something of value. Later life could therefore involve both loss and gain.

Migration added a further dimension to this process. Participants' accounts of possessing 'double assets' suggest what may be described

as a double horizon of experience: life could be interpreted through comparisons between past and present, country of origin and Sweden, hardship and security, and exclusion and belonging. Migration was not invariably experienced as strengthening, nor did earlier adversity necessarily produce resilience. For some participants, however, migration, adaptation, and hardship were retrospectively understood as experiences that had broadened perspective, strengthened confidence in coping with change, or deepened appreciation of later life. Migration could therefore become part of the interpretive resources through which meaning was sustained.

Empirical research broadly supports the relevance of meaning to well-being in later life. A meta-analysis (56) found meaning in life to be associated with fewer mental health problems, while a cross-sectional study (57) reported an association with resilience. More directly comparable, Borglin and colleagues' Swedish qualitative study of people aged 80 years and older found that quality of life involved a preserved self and meaning in existence, supported by continuity across past, present, and future (58). The present findings suggest that, in the context of migration, such continuity may also need to be negotiated across countries, languages, and different social worlds.

Frankl's existential theory (59) helps illuminate why meaning emerged so centrally in the material. His work emphasises that meaning may be found through commitments, relationships, responsibilities, and the way a person responds to circumstances that cannot always be changed. This resonates with participants' accounts of work accomplished, sacrifices made, children raised, relationships sustained, and hardships retrospectively incorporated into a meaningful life story. Importantly, this does not imply that adversity was inherently meaningful or beneficial, but that participants could sometimes assign meaning to experiences that had already occurred and integrate them into their understanding of the life they had lived. Martela and Steger's distinction between coherence, purpose, and significance (60) further helps specify what such meaning involved. Coherence was reflected in efforts to understand how earlier

experiences, migration, ageing, and the present fitted together as parts of one life; purpose in valued roles, commitments, relationships, and reasons to remain engaged in everyday life; and significance in the sense of having contributed, mattered to others, and lived a life of value. Meaning was therefore not an abstract concern, but involved being able to understand one's life as a whole, retain direction in the present, and experience oneself as someone whose life continued to matter.

While Frankl (59) and Martela and Steger (60) help illuminate why meaning matters and what it may consist of, Weick's concept of sensemaking (61) provides a process-oriented perspective on how such meaning is constructed and maintained under changing circumstances. Sensemaking is not a fixed interpretation reached once and for all, but an ongoing process in which people notice and interpret events in relation to previous experience and their sense of who they are, constructing plausible accounts that provide coherence and orientation. These interpretations can in turn guide how present circumstances are approached and may be revised as new experiences occur. In the present study, participants repeatedly revisited migration, family, belonging, bodily change, and anticipated dependency in light of one another. Past experiences were thus not simply remembered but reinterpreted from the standpoint of the present, while these interpretations also shaped how participants understood themselves and approached the future. Sensemaking therefore helps explain how continuity could be maintained without requiring life circumstances, or understandings of them, to remain unchanged.

Taken together, the findings suggest that the quest for meaning in later life involved both continuity across time and the reinterpretation of accumulated experience. Migration could complicate this process through disruption and non-recognition but could also broaden the perspectives and resources through which life was understood. Ageing in the context of migration may therefore involve continually

reworking the meaning of a life lived across more than one social world.

5.2 Family, social connectedness, and care as relational arenas of ageing

Across Studies II–IV, family, social relationships beyond the family, and anticipated or actual care emerged as important relational settings in which participants were concerned not only with receiving practical support, but with remaining connected to others in ways that sustained belonging, recognition, and continuity of self.

Family was a central relational context of ageing. Through roles as spouse, parent, and grandparent, participants remained connected to earlier parts of their lives and to a sense that they continued to matter to others. Family therefore provided not only support, but continuity, belonging, and a sense of accomplishment across generations. At the same time, family relationships contained an important tension. Participants wanted closeness and emotional support while generally wishing to avoid becoming a burden on their children. The acceptability of support therefore depended not only on the help provided, but also on whether it could be received without undermining reciprocity, independence, or self-respect. Family was consequently both a source of security and a setting in which dependence had to be morally negotiated.

Beyond the family, the findings show that social connectedness depended on more than contact or participation. Participants sought relationships in which they could understand others and, importantly, be understood themselves. Jenkins's account of identity as socially negotiated (62) is useful here, because continuity of self depends partly on how a person is recognised by others. This helps explain why participants could be socially active yet still experience distance or conversely maintain a relatively small social network while describing meaningful belonging. Selectivity in relationships could therefore

represent not withdrawal, but an effort to invest in relationships characterised by mutual understanding and reciprocity. Research on older migrants similarly suggests that belonging depends not only on the availability of social ties, but on their quality and on opportunities for reciprocal and meaningful participation (63).

Care constituted a further relational setting in which recognition became particularly important. Participants generally expressed trust in Swedish healthcare and eldercare, but their reflections on future dependency centred on whether care would enable them to remain themselves in familiar and recognisable ways. Communication, continuity, trust, and attentiveness to personal history therefore mattered alongside practical and clinical care. Research on migrant care experiences likewise indicates that linguistic and cultural barriers may hinder communication and participation, sometimes leading relatives to assume roles as interpreters or mediators (12, 64). While family involvement may be valuable, equitable formal care cannot depend on relatives compensating for shortcomings in communication or person-centred practice.

Across these settings, what mattered was not support alone, but whether relationships enabled participants to remain connected, understood, and recognised as people with continuing histories, identities, and roles. These conditions became particularly important when greater reliance on others was anticipated.

5.3 Sustaining dignity when independence becomes uncertain

Study IV suggests that what changed as participants anticipated increasing dependency was not whether dignity mattered, but how dignity could be sustained. While participants expected to remain able to think clearly, make decisions, and manage everyday life, dignity was closely associated with autonomy, control, and having one's wishes respected. Their concerns about cognitive decline therefore

extended beyond functional loss to the prospect of losing the ability to express preferences, make oneself understood, and influence how one was treated. At the same time, dignity was not understood solely as something maintained through individual agency. As personal control became less certain, its preservation increasingly depended on trustworthy relationships and care environments. What appeared most threatening was therefore not dependency as such, but dependency without trustworthy support.

Nordenfelt's theory of dignity provides a useful way of interpreting this tension. His typology distinguishes, among other forms of dignity, between *universal human dignity* (*Menschenwürde*) and *dignity of identity* (65). Universal human dignity belongs equally to all human beings and cannot be lost through illness, dependency, or diminished cognitive capacity. Dignity of identity, by contrast, concerns the integrity of the person, including self-respect, self-image, and how the person is recognised and treated by others. It is therefore more vulnerable to changes in health and capacity as well as to the conduct of other people.

This distinction helps clarify participants' concerns about what was described in Study IV as a *cognitive threshold*. Their accounts should not be understood as suggesting that dementia entails a loss of human worth. Rather, they point to a feared stage at which the person may become less able to communicate preferences, resist unwanted treatment, or protect dignity of identity independently. The ethical significance of the cognitive threshold therefore lies not in the disappearance of dignity, but in a shift in responsibility for safeguarding its expression in everyday life. Research on dignity in dementia supports this relational interpretation, showing that dignity may be sustained through personalised and attentive care and undermined through objectification, misunderstanding, or treatment that reduces the person to a set of care needs (66–67).

Seen in this way, several findings from Study IV acquire greater significance. Home, familiar language, established routines,

meaningful activities, and continuity in relationships were not merely matters of comfort. They could support dignity of identity by preserving continuity between the person one had been and the person one remained. Conversely, technically adequate and physically safe care could still threaten dignity if the person became misunderstood, depersonalised, or disconnected from familiar ways of being. The participants' concern was therefore not simply whether they would receive care, but whether they could continue to be cared for as themselves.

The categories *letting go* and *surrendering with trust* further complicate an understanding of dignity based primarily on independence. Letting go could involve actively revising expectations or accepting forms of control that were no longer possible, while retaining self-respect and coherence. Surrendering with trust, in turn, did not mean surrendering dignity itself, but entrusting more of its safeguarding to others. This is consistent with relational understandings of autonomy (42), in which autonomy is not equated with self-sufficiency but may be supported through relationships in situations of vulnerability. Dependence may therefore threaten dignity when it entails loss of voice, recognition, or trustworthy support, but it need not be undignified in itself.

Nordenfelt's broader theory of health (32–33) adds a further dimension to this interpretation. If health concerns a person's ability, under the circumstances in which life is lived, to realise vital goals, then declining capacity does not make relationships and care environments merely external to health. They may become part of the conditions that determine whether valued goals remain attainable. In the present findings, such goals could include remaining connected to family, making everyday choices, being understood in a familiar language, continuing meaningful activities, or preserving familiar ways of living. Supportive care may therefore help sustain the possibility of pursuing what still matters even when individual capacities diminish. This does not mean that health and dignity are the same: declining health does not diminish universal human

dignity. Rather, Nordenfelt's two perspectives together help show how care can both support the pursuit of vital goals and safeguard dignity of identity when increasing dependency changes what the person can accomplish alone.

5.4 Culture and language without essentialising difference

The findings show that culture and language could matter considerably in later life, particularly when participants needed to express preferences, emotions, personal histories, or ways of living that were important to them. This became especially relevant when dependence increased and communication with others became more consequential. Language was therefore not only a means of transmitting information, but also a means of expressing nuance, familiarity, humour, values, and biography. At the same time, being understood involved more than sharing a language; it also depended on whether others could recognise the person's history, values, relationships, and ways of being.

Hall's perspective (68) helps interpret these findings because it understands culture not as a fixed collection of traits belonging to particular groups, but as a system of meanings that is continually produced and negotiated. From this perspective, language forms part of how people communicate these meanings and make their experiences understandable to others. The importance some participants attached to a familiar or first language should therefore not be understood simply as a preference for linguistic convenience. In some situations, particularly in care, the ability to communicate in a familiar language could help people express aspects of themselves that might otherwise be difficult to convey. Language could therefore provide an important condition for understanding without, in itself, guaranteeing that understanding would occur.

At the same time, the findings do not support using culture as a general explanation for behaviour or care preferences. Ideas concerning family responsibility, autonomy, ageing, respect, and care varied both between and within the language groups and could change across generations and circumstances. Variation within groups was therefore as important analytically as variation between them. This is consistent with perspectives that understand culture as dynamic and context-dependent rather than as a fixed set of characteristics attached to particular groups (69).

Hall's concept of representation (68) further draws attention to how social categories acquire meanings beyond their descriptive function. In the present material, being perceived as an immigrant sometimes remained socially consequential after decades in Sweden, but its importance differed considerably between participants and situations. The category could position a person as different or foreign even when that person regarded Sweden as home and expressed strong trust in Swedish society. Finnish-speaking participants, for example, described this categorisation differently from some Chinese- and Arabic-speaking participants. Migration background was therefore neither irrelevant nor uniformly important.

Taken together, the findings suggest that culture and migration background are better understood as potentially significant contexts of ageing than as fixed explanations of individual preferences or behaviour. Their significance was situational and had to be understood in relation to language, biography, relationships, and circumstances. The analytical challenge is therefore neither to disregard migration-related differences nor to presume their meaning in advance.

5.5 Implications for person-centred care in later life

The findings suggest several implications for person-centred care in later life. These are relevant not only in services specifically directed towards migrants, but across eldercare and healthcare more broadly.

First, care should attend not only to symptoms, function, and practical needs, but also to what makes life meaningful for the individual. This includes valued roles, important relationships, fears concerning future loss, and activities or responsibilities that contribute to a sense of purpose. Supporting valued roles as a parent, grandparent, spouse, or contributor may therefore be important alongside addressing functional limitations.

Second, communication should be understood as central to autonomy and dignity rather than as a merely technical matter. Attention to preferred language, access to professional interpretation when needed, and sufficient time for dialogue can help people participate in decisions and express preferences, values, and concerns. Communication difficulties may otherwise increase the risk that the person is spoken over, misunderstood, or reduced to care needs rather than encountered as an individual.

Third, continuity can support both trust and continuity of self. When professionals know the person, understand their history, and recognise what matters to them, care is less likely to become impersonal or task-oriented. Continuity therefore concerns not only organisational consistency, but also whether biography, preferences, relationships, and established ways of living remain visible when care needs increase.

Finally, culturally responsive care requires attention to difference without assuming difference in advance. This accords with critiques of culturally prescriptive approaches, which caution against reducing patients to presumed group characteristics and instead emphasise dialogue and attention to the individual's own account (40,70). Older

foreign-born adults cannot be presumed to share particular preferences concerning family involvement, institutional care, autonomy, or ageing simply because of migration background. Language, migration history, family relationships, and cultural meanings may be highly important, but their significance needs to be explored through dialogue with the individual rather than inferred from group membership. The task for professionals is therefore not primarily to acquire fixed knowledge about how particular cultural groups are expected to think or behave, but to remain attentive to circumstances that may matter and explore their meaning with the individual.

Taken together, the findings suggest that person-centred later-life care requires attention not only to clinical safety and function, but also to communication, continuity, meaningful roles and relationships, and what matters to the individual.

5.6 Methodological reflections and limitations

This thesis was designed to generate conceptual understanding rather than statistical representation. The sequential grounded theory design allowed emerging findings from Phase I to inform subsequent sampling and data collection and enabled increasingly focused conceptual development across the four studies. The sample included variation in language background, migration history, age at migration, and life circumstances, but was not intended to represent particular ethnic or cultural groups. Most participants had lived in Sweden for many years, and people with more recent migration histories, severe frailty, or limited social participation were less well represented. The findings are also situated within Swedish welfare, eldercare, and migration contexts and should therefore be transferred to other settings conceptually rather than assumed to apply directly.

Researcher pre-understanding was another methodological consideration. Clinical experience in geriatric care, specialist nursing

education, personal experience of migration, and familiarity with Swedish health and welfare institutions may have increased sensitivity to issues relevant to the study but could also have directed attention towards already familiar concerns. To reduce the risk of theoretical forcing, engagement with substantive theories was delayed during early analysis, while constant comparison, memo-writing, attention to variation and contrasting cases, and regular discussions with the supervisory team were used to keep conceptual development grounded in the data.

Language mediation enabled participation across different levels of Swedish proficiency but also formed part of the interpretive process. Two Phase I interviews were interpreted by the same authorised professional interpreter, who was also a relative of both participants. This dual role may have influenced what was disclosed and how it was expressed (71–72). One focus group was interpreted by a professional Arabic interpreter, whereas the Mandarin-language material was collected directly by the author and translated into Swedish during transcription. These arrangements increased linguistic accessibility but created different conditions for data generation and interpretation. Conducting several discussions in participants' preferred languages nevertheless allowed greater scope for expressing nuance, biography, emotion, and values.

Recruitment procedures also influenced the composition of the sample and the development of the study. Initial attempts to recruit participants through municipal social centres did not result in any participants. Although this absence was not treated as an empirical finding in itself, it raised questions about which older adults such meeting places reached and whether some older foreign-born adults might remain outside these social arenas. These observations informed subsequent recruitment strategies and contributed to the further exploration of social connectedness and isolation. Recruitment was therefore broadened to include community organisations and social networks. These routes facilitated access to participants from different language backgrounds but may also have

favoured individuals who were more socially connected and introduced gatekeeper effects. The Arabic-speaking participants were recruited through contact with a Syrian Orthodox church. Religion was not a sampling criterion, and the church functioned as a point of access rather than as a basis for selecting participants according to faith. Nevertheless, this recruitment route may have shaped the composition of the two Arabic-speaking focus groups and limited the range of experiences represented within the Arabic-speaking material. The findings from this part of the material should therefore not be understood as representative of Arabic-speaking older adults more broadly.

Mini-focus groups of three to six participants were used to facilitate participation and discussion across language groups and around potentially sensitive topics. Their smaller size provided greater space for individual contributions but may have generated less interactional complexity than larger groups. One older Chinese-speaking participant chose an individual interview rather than group participation. Her account provided a useful comparative perspective but lacked the interactional material generated in focus groups and was therefore treated as supplementary.

The analysis was guided by Glaser's criteria of fit, work, relevance, and modifiability (43). Fit was pursued through constant comparison and grounding categories in the data; work and relevance concerned whether the concepts accounted for recurring concerns across the studies; and modifiability recognises that the resulting substantive theory remains open to revision. Further research involving more recently arrived, socially isolated, or frail older adults, and longitudinal research following changes in health and care needs, would provide important opportunities to extend or modify the conceptual framework.

Generative AI tools, including ChatGPT and Microsoft Copilot, were used during preparation of the thesis for language refinement, stylistic editing, and support with clarity and structure. All analytical

interpretations, research decisions, source verification, and final responsibility for the text remained with the author.

5.7 Conclusion

This thesis suggests that ageing among older foreign-born adults in Sweden can be understood as *a quest for life and its meaning*: an ongoing effort to make sense of life across past, present, and future while sustaining meaningful relationships and dignity under changing circumstances. Across the four studies, this quest was synthesised as an ongoing effort to preserve a meaningful self.

The findings do not suggest that older foreign-born adults experience fundamentally different concerns from other older adults. Rather, they show how concerns common to later life—meaning, belonging, recognition, autonomy, and dignity—may take particular forms when lives have unfolded across migration histories, languages, and more than one social world.

The contribution of the thesis lies in clarifying some of the conditions through which a meaningful and dignified later life may be sustained in the context of migration. For gerontological nursing and person-centred care in later life, the findings highlight the importance of understanding what matters to the individual and of supporting continuity of self as health, independence, and care needs change. This includes attention not only to safety and function, but also to communication, meaningful roles and relationships, and the person's life history, preferences, and ways of being. As personal agency becomes more limited, the responsibility of others for safeguarding dignity in everyday life may become increasingly important, by supporting the person to remain understood, recognised, and involved as far as possible.

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